

Willowbrook Medical Center

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Department of Internal Medicine

HOSPITAL DISCHARGE SUMMARY

This is a fictional, computer-generated patient record created for software testing and demonstration purposes (e.g. RAG / document Q&A pipelines). It does not describe a real person and should not be used for any clinical, billing, or research purpose.

Patient Name	Michael J. Rodriguez	Medical Record No.	00-45-7823
Date of Birth	03/14/1968	Age / Sex	58 years / Male
Admission Date	08/10/2026	Discharge Date	08/14/2026
Attending Physician	Sarah Chen, MD	Service	Internal Medicine
Admitting Diagnosis	Community-acquired pneumonia	Code Status	Full Code

VITAL SIGNS ON ADMISSION

Temp	Heart Rate	Blood Pressure	Resp. Rate	SpO2 (RA)
101.8°F (38.8°C)	108 bpm	148/92 mmHg	24/min	89%

CHIEF COMPLAINT

"I've had a fever and cough for the past five days, and I'm having trouble catching my breath."

HISTORY OF PRESENT ILLNESS

Mr. Rodriguez is a 58-year-old male with a past medical history of type 2 diabetes mellitus, hypertension, and hyperlipidemia who presented to the Emergency Department with a five-day history of productive cough with yellow-green sputum, subjective fevers, chills, pleuritic right-sided chest pain, and progressively worsening dyspnea on exertion. He denied hemoptysis, recent travel, or sick contacts. He reported poor oral intake over the preceding two days and was found to have elevated home glucose readings (240–280 mg/dL) in the days prior to admission. On arrival he was febrile, tachycardic, and hypoxic to 89% on room air, prompting admission for further management.

PAST MEDICAL & SURGICAL HISTORY

Type	Details
Medical	Type 2 Diabetes Mellitus (dx. 2015), Hypertension (dx. 2012), Hyperlipidemia, Osteoarthritis (right knee)
Surgical	Appendectomy (1990); Right knee arthroscopy (2019)

HOME MEDICATIONS (PRIOR TO ADMISSION)

Medication	Dose	Route	Frequency
Metformin	1000 mg	PO	Twice daily
Lisinopril	20 mg	PO	Once daily

Medication	Dose	Route	Frequency
Atorvastatin	40 mg	PO	Once daily (evening)
Aspirin	81 mg	PO	Once daily

ALLERGIES, SOCIAL & FAMILY HISTORY

Allergies	Penicillin (hives)	Tobacco Use	Former smoker, quit 2010 (20 pack-yr)
Alcohol	Occasional, 1–2 drinks/week	Occupation	Retired construction worker
Living Situation	Lives with spouse	Family History	Father: MI at age 65. Mother: Type 2 DM.

PHYSICAL EXAMINATION (ON ADMISSION)

System	Findings
General	Ill-appearing, mild respiratory distress, oriented x3
HEENT	Normocephalic, atraumatic; mucous membranes dry
Neck	Supple, no jugular venous distension, no lymphadenopathy
Cardiovascular	Tachycardic, regular rhythm, no murmurs/rubs/gallops
Pulmonary	Coarse crackles at right lower lobe; decreased breath sounds RLL; mild use of accessory muscles
Abdomen	Soft, non-tender, non-distended, normoactive bowel sounds
Extremities	No cyanosis, clubbing, or edema
Neurological	Alert and oriented x3, no focal deficits
Skin	Warm, dry, no rashes

LABORATORY RESULTS (ADMISSION)

Test	Result	Reference Range	Flag
White Blood Cell Count	14.2 x10 ³ /uL	4.5 – 11.0 x10 ³ /uL	HIGH
Hemoglobin	13.1 g/dL	13.5 – 17.5 g/dL	LOW
Hematocrit	39.4 %	41 – 53 %	LOW
Platelets	245 x10 ³ /uL	150 – 400 x10 ³ /uL	Normal
Sodium	136 mEq/L	135 – 145 mEq/L	Normal
Potassium	4.2 mEq/L	3.5 – 5.0 mEq/L	Normal
Chloride	100 mEq/L	98 – 106 mEq/L	Normal
Bicarbonate (CO2)	24 mEq/L	23 – 29 mEq/L	Normal
BUN	18 mg/dL	7 – 20 mg/dL	Normal
Creatinine	1.0 mg/dL	0.7 – 1.3 mg/dL	Normal

Test	Result	Reference Range	Flag
Glucose (random)	218 mg/dL	70 – 140 mg/dL	HIGH
Hemoglobin A1c	8.2 %	< 5.7 %	HIGH
Procalcitonin	2.1 ng/mL	< 0.10 ng/mL	HIGH
Lactate	1.8 mmol/L	0.5 – 2.2 mmol/L	Normal

IMAGING STUDIES

Chest X-ray (PA/Lateral, 08/10/2026): Focal airspace consolidation in the right lower lobe consistent with pneumonia. No pleural effusion or pneumothorax. Cardiomeastinal silhouette within normal limits.

HOSPITAL COURSE

Mr. Rodriguez was admitted to the general medicine floor with a diagnosis of community-acquired pneumonia complicated by hypoxia and poorly controlled type 2 diabetes. He was started on empiric intravenous ceftriaxone 1 g every 24 hours and oral azithromycin 500 mg daily, and placed on supplemental oxygen via nasal cannula with a goal SpO₂ > 92%. Blood and sputum cultures were obtained; blood cultures showed no growth at 48 hours and sputum culture was consistent with oropharyngeal flora. Procalcitonin trended down over the admission, supporting a bacterial source that was responding to therapy.

Glycemic control was managed with correctional insulin (sliding scale) while home metformin was held during the acute illness. Endocrinology was consulted given an elevated HbA1c of 8.2%, and recommended initiation of a second oral agent at discharge. The patient's oxygen requirement resolved by hospital day 3, and he was afebrile for more than 48 hours prior to discharge. He was transitioned to oral amoxicillin-clavulanate to complete a seven-day antibiotic course as an outpatient. He was ambulating independently and tolerating a regular diabetic diet at the time of discharge.

DISCHARGE DIAGNOSES

Type	Diagnosis	ICD-10
Primary	Pneumonia, right lower lobe, bacterial	J15.9
Secondary	Type 2 Diabetes Mellitus, poorly controlled	E11.65
Secondary	Essential Hypertension	I10
Secondary	Hyperlipidemia	E78.5

DISCHARGE MEDICATIONS

Medication	Dose	Route	Frequency	Notes
Amoxicillin-clavulanate	875 mg	PO	Twice daily x 7 days	NEW – complete course
Metformin	1000 mg	PO	Twice daily	Continue
Glipizide	5 mg	PO	Once daily, AM	NEW – glycemic control
Lisinopril	20 mg	PO	Once daily	Continue
Atorvastatin	40 mg	PO	Once daily, evening	Continue

Medication	Dose	Route	Frequency	Notes
Aspirin	81 mg	PO	Once daily	Continue

DISCHARGE INSTRUCTIONS

Diet: Consistent-carbohydrate (diabetic) diet, low sodium. **Activity:** Resume normal activity as tolerated; avoid strenuous exertion for one week. **Glucose Monitoring:** Check fingerstick blood glucose four times daily (before meals and at bedtime) and record results for review at follow-up. **Return Precautions:** Return to the Emergency Department for recurrent fever above 101°F, worsening shortness of breath, chest pain, confusion, or blood glucose readings persistently above 300 mg/dL or below 70 mg/dL.

FOLLOW-UP APPOINTMENTS

Provider	Specialty	When	Reason
Amanda Torres, MD	Primary Care	08/21/2026 (1 week)	Post-hospitalization follow-up
Endocrinology Clinic	Endocrinology	Within 3–4 weeks	Diabetes management, adjust regimen

DISCHARGE CONDITION

Stable and improved. Afebrile, oxygen saturation 96% on room air, tolerating oral intake and ambulating independently at time of discharge.

Electronically signed by:

Sarah Chen, MD — Attending Physician, Internal Medicine

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